MEDCONTRACTINTEL

VOL. III · HOSPITALIST

Volume III of the MedContractIntel Compensation Series

Hospitalist Shift Economics

The Math Behind 7-on/7-off, Nocturnist Pay, Admission Fees, and the Career Gap Nobody Talks About

Data. Leverage. Fair Pay.

MedContractIntel Compensation Series · Volume III
Internal Medicine · Hospitalist Track

Internal Medicine Edition · 2026

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How to use this book

Hospitalist Shift Economics is structured to be read in order or used as a reference. Each chapter stands on its own, but the chapters build on each other. Here's where to look for what you need most.

Read the Foreword first.

It explains why hospital medicine is the specialty where the information gap hits hardest, and what unique levers hospitalists have that outpatient IM physicians don't. Five-minute read.

Use Part I to compute your effective hourly rate.

If you can't state your effective hourly rate from memory, you can't negotiate. Part I gives you the schedule math (182 working days, 2,184 patient-facing hours) and the formula.

Use Part II if you do nights.

Nocturnist compensation is the single most under-negotiated term. Part II shows the 15–25% differential range, the math on a $280K base, and the schedule-vs-compensation differential trap.

Use Part III to understand the hospital's revenue.

When the conversation turns to budget constraints, knowing the MS-DRG side of the ledger gives you factual framing without going adversarial.

Use Part IV to find your percentile.

Then Part V tells you which five contract terms to negotiate first. Three things to do before you sign close out the book.

Pair this with the Negotiation Script Pack (Volume II).

Script 5 in the Pack is the word-for-word nocturnist conversation that uses the math from Part II of this book.

Scan icons

The body uses four small marks to help you skim. Each labels what kind of paragraph it leads.

■	**DATA**	A key benchmark or specific dollar figure you can cite
▲	**WATCH**	A common employer trap or below-market structural pattern
★	**WIN**	A move that compounds in your favor over the contract life
■	**DO**	A concrete action — pull this number, request this clause, do this thing

Cross-references to other titles in the MedContractIntel Compensation Series (the Internal Medicine Physician Contract Red Flag Checklist, the Negotiation Script Pack, the wRVU Playbook, and the Hospitalist Shift Economics Analyzer) appear in the body where relevant. Each title is independent but designed to complement the others.

Foreword

The Information Gap

You completed residency. You are running a hospital service. You are managing 15 to 20 admissions. Someone handed you a contract that says "$280,000 base" or "$2,200 per shift" or "$320,000 productivity target." You signed it, or you are about to.

What you do not know: what the hospital collects on those same admissions. What the math actually looks like across a full schedule year. What "7-on/7-off" means in terms of billable encounters per year versus 365 days of availability. What the difference is between your annual income at the 25th versus the 75th percentile of published 2025 hospitalist compensation data. This book covers all of it.

Hospital medicine is the specialty where the information gap hits hardest. Emergency Medicine physicians have a clear per-shift or per-hour rate structure — the math is transparent. Outpatient Internal Medicine has wRVU-denominated production structures that, while complex, at least put the numbers on the page. Hospital medicine occupies an uncomfortable middle ground: shift-based work with production features, production-based work with shift constraints, and a compensation structure built from three or four overlapping components — base, production, nocturnist or shift differential, admission fee, quality bonus — that do not obviously add up to a simple per-shift or per-hour rate.

This opacity is not accidental. A hospitalist who does not understand the component math of their own compensation cannot negotiate effectively. A hospitalist who does understand the math can walk into any negotiation and say: *"My effective hourly rate across a 2,184-hour year at current compensation is $X. Published 2025 surveys place the median for Hospitalist: Internal Medicine total compensation around $320,000, which translates to an hourly rate near $147. My hourly rate is below median by $W. That is the conversation I want to have."*

This book gives you that math. Five parts: the schedule math, the nocturnist differential math, the revenue math on the hospital side, the benchmark math, and the five contract terms that move the number most. Plus a closing action section of three things to do before you sign. Read it in order. Each section builds on the one before.

~$320K

Median Hospitalist: IM Total Compensation

Published 2025 surveys, synthesis range

One final note before starting. Nothing in this book is adversarial toward hospitals. Hospitals are complex institutions with substantial overhead and real margin pressures. The point of understanding the economics is not to extract maximum compensation at the employer's expense. The point is to ensure that your compensation reflects the market rate for your clinical service, rather than defaulting to whatever number the organization proposes. A well-negotiated contract is good for both parties. A poorly negotiated contract is good for neither — it just takes longer for the damage to become visible.

Part I — The Schedule Math

The 7-on/7-off Schedule

The 7-on/7-off schedule is the dominant hospitalist work structure in the United States. It is also routinely misunderstood in terms of annual hours, annual shifts, and effective hourly compensation. The math is straightforward, but it is almost never presented clearly.

Basic annual shift calculation

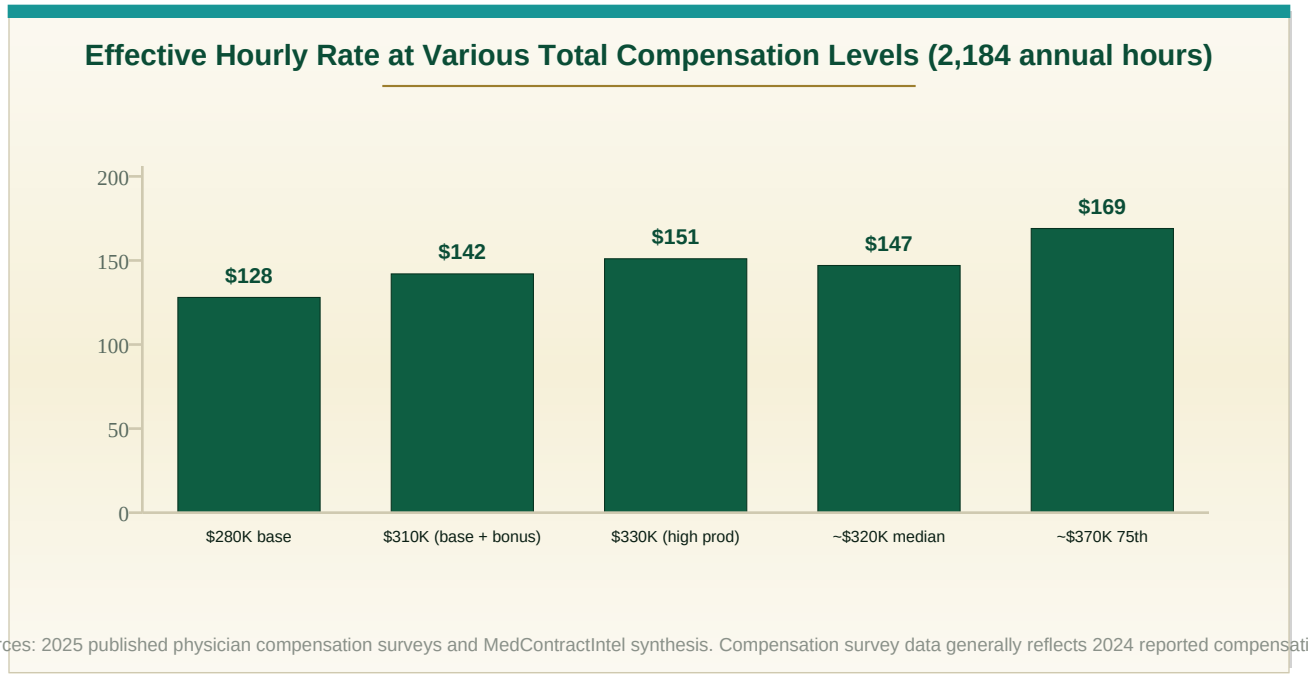
A 7-on/7-off schedule produces 26 two-week blocks per year (52 weeks ÷ 2 = 26). Each block is 7 working days and 7 off days. Total working days per year: 26 × 7 = 182. This means a hospitalist on true 7-on/7-off works roughly half the year.

Shift length and annual hours

Median hospitalist shift length is 12 hours, with median annual patient-facing hours around 2,040. If shifts are 12 hours, 182 working days produces 2,184 patient-facing hours. The difference between 2,184 and 2,040 reflects variation in actual shift length, vacation or swap coverage, and non-shift administrative time that counts toward the annual total.

Effective hourly rate calculation

This is the number that tells you whether your compensation is competitive on a time-adjusted basis. Take total annual cash compensation, divide by 2,184 hours, and you have your effective hourly rate.



These numbers are useful in two ways. First, they let you compare hospitalist compensation to other specialty and clinical roles on a time-adjusted basis. Second, they let you calculate the opportunity cost of accepting additional shifts or declining locum work.

Admission volume and census dynamics

If your service runs 12 admissions per day on your 7-day block, by Day 7 you have either handed off or retained up to 84 patient-days of care. Your actual census depends on average length of stay — at a typical 4-day ALOS, your rolling census settles around 20–25 patients by mid-block. The cognitive and workload reality of Day 5, Day 6, Day 7 of a full census block is not the same as Day 1 of a ramping census. Your per-shift productivity is not constant across the week; it is highest when you are picking up stable census, lowest when admissions and discharges are both heavy.

Per-admission fee structures

■ Some hospitalist groups pay a flat per-admission fee on top of base — typically $50 to $150 per admission, with $100–$125 being common. Whether this is beneficial depends on admission volume. At 400 admissions per year (typical for a full-time hospitalist on a general medicine service), a $100/admission fee adds $40,000 to annual compensation. At 250 admissions per year, only $25,000. The per-admission fee concentrates compensation on high-volume shifts and may under-compensate high-acuity work, where fewer but sicker admissions dominate the day.

The Day 1 versus Day 7 weighting problem

Day 1 of a block is admission-heavy: you are picking up the previous team's hand-offs, doing your own new admissions, and writing initial hospital care notes (99223 at 3.50 wRVUs each). Day 7 is discharge-heavy: you are handing off to the incoming team, finalizing discharge summaries, and writing

discharge codes (99238 at 1.50 wRVUs; 99239 at 2.15 wRVUs). If your compensation is on a wRVU-production model, your Day 1 shifts generate substantially more wRVUs than your Day 7 shifts. This is structural, not strategic — it is how the code values line up against the service rhythm.

The practical implication: if your group is considering a structural shift from base-plus-bonus to pure production, evaluate how your personal shift pattern loads admissions versus discharges. A hospitalist on an admitter-heavy rotation benefits from pure production; a hospitalist on a rounder-heavy or discharge-heavy rotation is penalized by it.

Part II — The Differential Math

Nocturnist Pay

Nocturnist compensation is the single most commonly under-negotiated term in hospitalist contracts. The pattern is consistent: a differential is offered that sounds reasonable in isolation, the physician accepts it without benchmarking, and the differential is well below the published median by the time the physician realizes it.

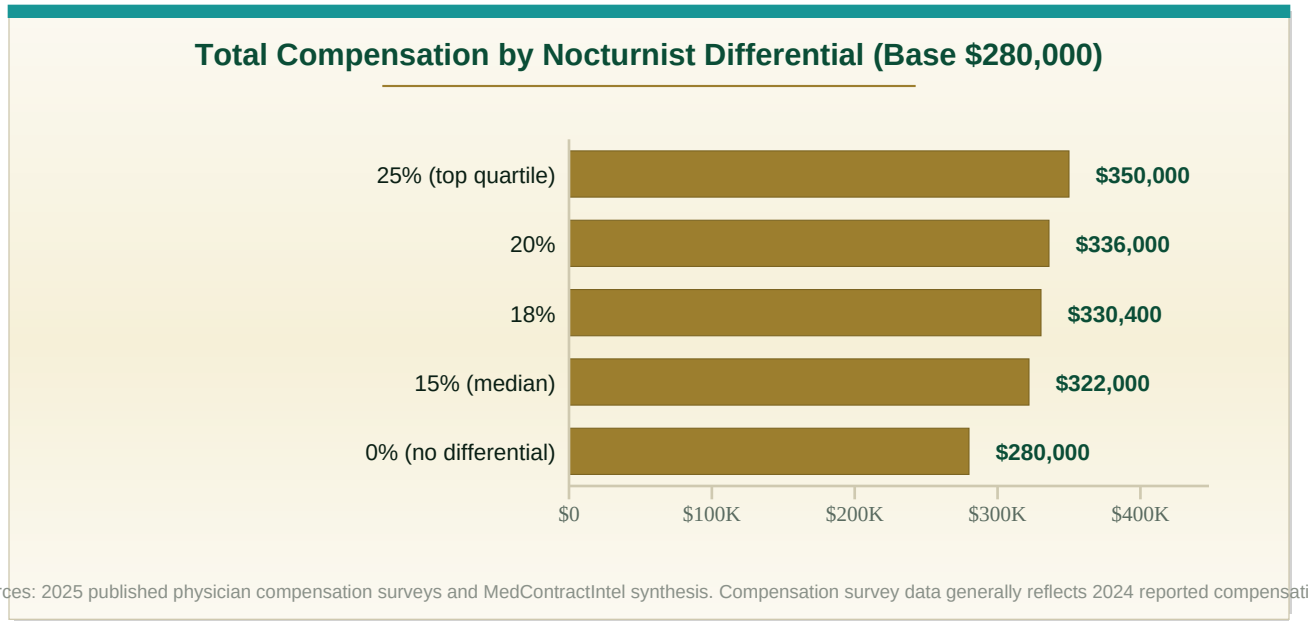
What "nocturnist" means in practice

Night shift — typically 7 PM to 7 AM — with variable admission responsibilities depending on group structure. In some groups, nocturnists handle all nighttime admissions with no planned admissions for daytime teams; in others, the nocturnist handles cross-coverage only, with day-team admissions absorbed into the morning service. Scope matters for compensation: a nocturnist handling 8–12 admissions per night has a different workload than one handling cross-coverage for a stable census.

The benchmark range

■ Among hospitalist groups that pay any nocturnist compensation differential, the typical differential runs **15–25%** above base hospitalist compensation. The midpoint (15–20%) reflects the most common structures; top-quartile groups pay 20–25%. Roughly two-thirds of groups that employ nocturnists pay any compensation differential at all — the remaining third either pay no differential or provide a schedule differential instead (fewer shifts worked for the same pay).

The math of differential percentages on a $280,000 base



The difference between a 15% and 20% differential is $14,000 per year. Over a five-year term, that is **$70,000**. The difference between a 15% and 25% differential is $28,000 per year, or **$140,000** over five years. A nocturnist who accepts an "industry-standard" 15% differential without negotiating is leaving real money on the table relative to the top-quartile market.

$140,000

5-Year Cost of a 10-Point Differential Shortfall

25% vs. 15% differential on $280K base

The "getting used to nights" trap

▲ Some employers rely on the adaptation of nocturnists to the schedule — the argument being that after six months, the physician has adjusted and no longer needs premium compensation. This is a compensation argument dressed as a physiology argument. Adapting to night work does not reduce the schedule's compensation value. The schedule is the schedule. The data is the data. Anchor to the data.

The correct opening

> *Published 2025 hospitalist compensation surveys place the typical nocturnist differential at 15 to 25 percent above base, with median structures clustering at 15–20 percent and top-quartile groups paying 20–25 percent. My current differential is [X]%. I'd like to discuss closing the gap to the median at minimum, or into the top quartile if the role's scope warrants it.*

The schedule-differential counter

If the group offers a schedule differential (fewer shifts worked at the same pay) instead of a compensation differential, calculate the equivalent percentage. If daytime hospitalists work 182 shifts per year and nocturnists work 164 shifts per year (approximately 10% fewer), that is roughly equivalent to an 11% compensation differential. Below 11%, the total package is below median. Above 15%, the package approaches the top quartile.

The one-time bonus counter

▲ A common employer move is to propose a one-time sign-on or retention bonus instead of a structural differential. Reject this. A $15,000 one-time bonus in year one is worth less than a structural 10% differential on a $280,000 base across a three-year term ($84,000). Compensate for the schedule, not for the signing.

Part III — The Hospital's Math

Two Revenue Streams

Every hospitalist encounter generates two revenue streams for the organization. Understanding both streams is essential for having the right conversation about compensation.

Stream 1: Professional fee

The payment to the physician for the clinical service. Billed through the physician's NPI on CMS-1500 claims. For a 99223 initial inpatient admission (3.50 wRVU), Medicare pays approximately **$156** at the 2026 non-QP conversion factor of $33.40. For a 99233 subsequent daily visit (2.40 wRVU), approximately **$107**. For a 99238 discharge (1.50 wRVU), approximately **$75**. These are the rates the physician's compensation model is calibrated against.

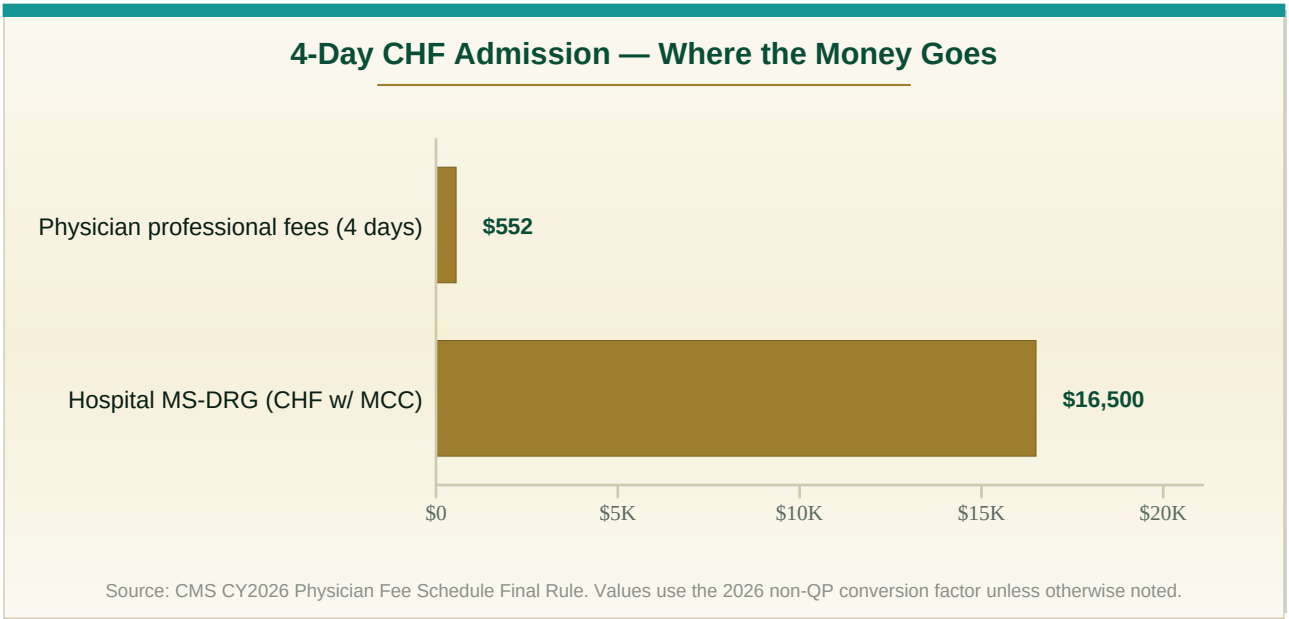
Stream 2: Facility fee / MS-DRG

The payment to the hospital for the hospitalization itself — room, nursing, pharmacy, diagnostics, labs, procedures, everything other than physician professional services. Billed through the hospital's NPI on UB-04 claims. The Medicare payment is determined by the MS-DRG (Medicare Severity Diagnosis Related Group) into which the admission classifies, adjusted for the hospital's wage index and specific payment parameters. MS-DRG payments for medical admissions typically range from approximately $8,000 to $25,000+, with specific ranges varying by DRG, severity, and hospital type.

For illustration: a community-acquired pneumonia admission without major comorbidities might generate an MS-DRG payment of approximately $8,000–$10,000. A pneumonia admission with complications (MCC) could generate $12,000–$16,000. A septicemia admission with multiple organ involvement could exceed $20,000. Congestive heart failure with MCC often generates $14,000–$18,000.

The strategic implication

When a hospitalist admits a patient with CHF exacerbation and MCC, the hospital's facility-side revenue on that admission may be $15,000–$18,000. The physician's professional-fee revenue on the same admission across a 4-day length of stay is approximately:



This is not a scandal. The MS-DRG payment covers substantial hospital overhead — nursing care, pharmacy, diagnostics, overhead. The point is not that hospitals are keeping money that should go to physicians. The point is that the physician's compensation conversation should be framed around the market rate for the physician's clinical service, not around whatever fraction of facility revenue the organization chooses to allocate to compensation.

Documentation integrity and the physician's role in facility revenue

The DRG assignment for an admission depends on the documented diagnoses. A hospitalist who thoroughly documents comorbidities and complications can shift a DRG from a lower-paying to a higher-paying category — not because of upcoding, but because of accurate documentation. The CHF admission documented as "CHF exacerbation" may classify to a lower DRG than the same admission documented as "acute on chronic systolic CHF exacerbation with acute kidney injury and hyponatremia." Both may be clinically accurate; the difference is the thoroughness of documentation.

In systems where hospitalist compensation includes a case-mix-index (CMI) bonus or a documentation integrity component, this work directly translates to physician compensation. Even where it does not, the CMI impact affects the organization's overall financial health and the sustainability of the hospitalist service.

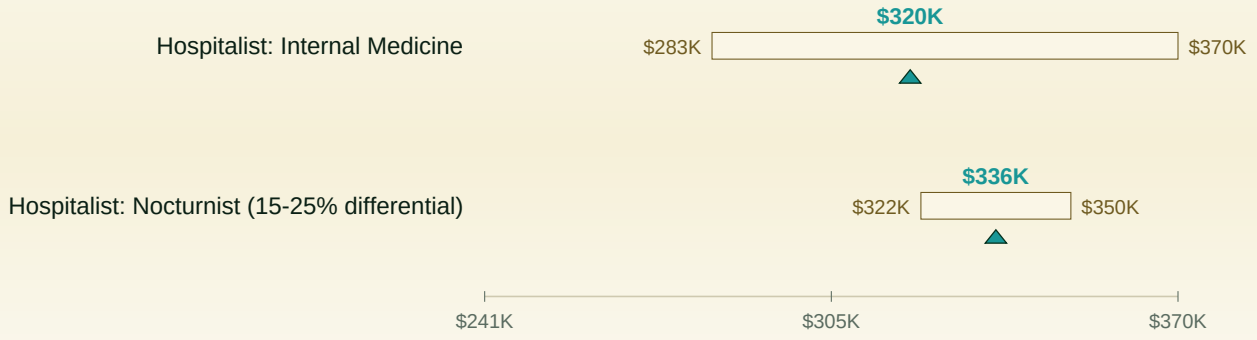
The point of understanding this data is not to develop adversarial framing. It is to have factual conversations about compensation anchored to the economics of the work, rather than to the compensation framework the organization defaults to.

Part IV — Your Position

Benchmark Yourself

Every hospitalist should know three numbers: their current total cash compensation, their current annual wRVU production, and their position on the 2025 published compensation curve. If you cannot state all three from memory, you cannot negotiate effectively.

Hospitalist: Internal Medicine — published 2025 reference range



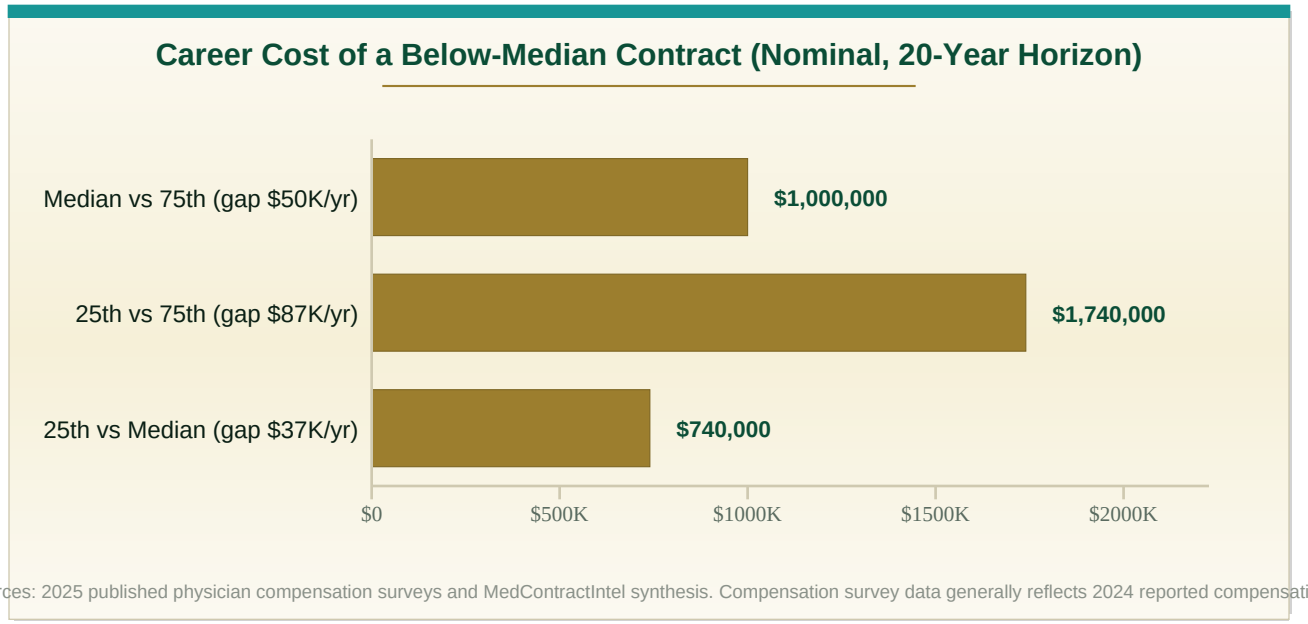
Sources: 2025 published physician compensation surveys and MedContractIntel synthesis. Compensation survey data generally reflects 2024 reported compensation.

How to find your position

Add up your actual total cash compensation for the most recent 12 months: base + productivity bonus + quality bonus + sign-on or retention bonus received + any other cash compensation. Compare to the chart above. If you are at $310,000, you are between the 25th percentile and median. If you are at $350,000, you are between the median and 75th percentile.

The compounding math of percentile differences

A hospitalist at the 25th percentile (~$283K) versus the median (~$320K) earns approximately $37,000 per year less. Over five years, $185,000. Over a 20-year career, more than $740,000 nominal — substantially more when compounded at typical investment returns.



$1.7M

Career Cost: 25th vs. 75th Percentile

Nominal, 20-year horizon

A hospitalist at the 25th percentile versus the 75th percentile (~$370K+) earns approximately $87,000 per year less. Over a 20-year career, more than $1.7 million nominal.

“The single highest-leverage financial decision most hospitalists will make is the decision of whether to negotiate the initial contract to market rates — or to accept below-market on the theory that the benefits make up for it.”

These are not theoretical numbers. They are the difference between a below-market contract and a market-rate contract, compounded over a career. The single highest-leverage financial decision most hospitalists will make is the decision of whether to negotiate the initial contract and early-career renewals to market rates, or to accept below-market compensation on the theory that "the benefits make up for it" or "negotiating is not done here."

Private-group versus hospital-employed hospitalists

A brief note on the structural difference. Hospital-employed hospitalists typically receive a base-plus-production structure with quality bonus, employer-provided malpractice (often occurrence coverage eliminating tail obligations), and formal benefits. Private-group hospitalists typically have higher cash compensation but carry more risk — claims-made malpractice with potential tail obligations, less structured quality bonus frameworks, and more variable productivity-based compensation. Self-reported survey means tend to run 5–10% higher than employer-reported medians because they over-represent self-employed and private-group physicians.

When benchmarking, use the dataset that matches your employment setting. Use ranges, not single point estimates — the data does not support more precision than that.

Part V — The Five Levers

Contract Terms That Move the Number Most

Five specific contract terms, in order of financial impact, for hospitalists. These are the terms that most meaningfully affect total compensation over the life of the contract. Focus negotiation energy on these five before anything else.

1. Base salary vs. production-only model

The choice of compensation structure has a larger effect on long-term earnings stability than any other contract term. A guaranteed base plus production-above-threshold is the most common and generally most favorable structure for a mid-career hospitalist. A pure production model is higher upside and higher variance; a pure salary model is the opposite. The specific term to negotiate is the threshold at which production bonus kicks in. A threshold near the median production benchmark (approximately 4,200 wRVUs for hospitalists) is reasonable; a threshold at the 75th percentile is structurally unreachable for median performers.

2. Nocturnist differential percentage

Covered in detail in Part II. The choice between a 15% and 20% differential on a $280,000 base is $14,000 per year — $70,000 over a five-year term. Anchor to the published 15–25% differential range and negotiate within it.

3. Shift guarantee minimum

▲ If your contract specifies a per-shift rate or a production-based compensation structure, the employer controls your income by controlling your schedule. A contract that does not specify a minimum number of annual shifts — typically 156–180 for a full-time hospitalist at 7-on/7-off — hands the employer complete flexibility to reduce your income without breaching the contract. Insist on a specific annual shift minimum.

4. Tail coverage responsibility on exit

▲ If your malpractice is claims-made and the contract does not specify employer-paid tail on separation, the default is that you pay the tail. For hospitalists, tail coverage typically costs $15,000–$40,000 depending on state and policy limits. Employer-paid tail on termination without cause is the most common negotiated compromise — the physician pays the tail only if they leave voluntarily or are terminated for cause. This is a single-term negotiation with substantial financial impact at the back end of employment.

5. Non-compete geography

A non-compete that excludes most of the accessible job market in your region is a substantial constraint on future leverage and mobility. Every year you work under a broad non-compete increases the cost of future negotiations, because the employer knows you cannot credibly threaten to leave for a competitor within reasonable distance. Negotiate the geographic scope down to the hospital(s) where you actually work, with a reasonable radius (5–10 miles in urban markets, 15–25 miles in less dense markets). Where state law voids non-competes (California, and increasingly with FTC-related state reforms), this is moot; elsewhere it is one of the most important mobility terms.

Other terms matter — CME allowance, PTO, sign-on bonus, clawback structure — but they are smaller dollar figures with less compounding effect. Spend negotiation capital on the five above first.

Closing

Three Things to Do Before You Sign

1. Pull your current or offered compensation against the published 2025 reference range

Take your total cash compensation. Compare to the Hospitalist: Internal Medicine reference range (approximately $283K to $370K+, median approximately $320K). Identify your position. If you are below the median and your production is not correspondingly below the median, you have a quantitative basis for a negotiation. If you are at or above median with commensurate production, your package is market-rate and the negotiation focus should shift to non-compensation terms.

2. Calculate your effective hourly rate across a full 182-day year

■ Take your total annual cash compensation. Divide by 2,184 (182 days × 12 hours) or by the specific number of hours your schedule actually produces. Compare to the implied hourly rates: at the median ~$147; at the 25th percentile ~$130; at the 75th percentile ~$169+. An hourly rate meaningfully below the specialty median is structural, not incidental — it reflects a below-market compensation structure that will not self-correct without negotiation.

3. Identify which of the 5 contract terms is most below-market in your specific offer

■ Review the offered contract against the five high-impact terms: compensation structure and threshold, nocturnist differential, shift guarantee minimum, tail coverage, and non-compete geography. Identify which is most problematic in your specific offer. Focus negotiation energy on the two or three most problematic, not on every term equally.

A contract negotiation is won or lost on the two or three most important terms, not on the fifteen or twenty total terms in the agreement. Know which ones matter most for your situation, and push hardest on those. The remainder will often fall into line once the major terms are settled.

BACK MATTER

Methodology & Sources

About the data. Compensation ranges in this guide are national synthesis ranges based on published physician compensation surveys and related industry sources available as of April 2026. Most published 2025 compensation surveys reflect 2024 reported compensation. CMS wRVU and Medicare payment references reflect CY2026 Physician Fee Schedule values.

Methodology. Compensation benchmarks reflect 2025 published surveys (Doximity, Medscape, MGMA). RVU and Medicare reimbursement values reflect CMS CY2026 PFS Final Rule, January 2026 release. Data last reviewed: April 2026.

Disclaimer. These materials are educational and informational only. They are not legal advice, financial advice, employment advice, a fair market value opinion, a Stark Law opinion, an Anti-Kickback Statute opinion, or a compensation consulting report. Individual contracts vary by geography, practice setting, payer mix, productivity, benefits, tenure, and employer structure. Consult a qualified physician-contract attorney and/or financial advisor before making contract decisions.

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MEDCONTRACTINTEL

Data. Leverage. Fair Pay.

Identify your contract's red flags before signing.
Benchmark your pay against current published
compensation surveys and CMS 2026 wRVU/payment values.
Walk into the conversation with the same numbers
the compensation analyst on the other side has.

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